THE MATERNITY METHOD

NCLEX CHEAT SHEET

Chorioamnionitis & Neonatal Sepsis

Maternal & Newborn › Intrapartum & Newborn

The Big Picture

Chorioamnionitis (also called intraamniotic infection) is infection of the **membranes and amniotic fluid**, usually from bacteria **climbing up from the vagina**, most often after membranes have been ruptured a long time. It can pass to baby and cause **newborn sepsis**, which is hard to spot because the signs are **subtle**.

KNOW THE NUMBERS

≥ 38.0 °C	> 160	> 18 hrs	≥ 4 hrs	< 72 hrs
Mom's **fever**: act on it	**Fetal HR**: tachycardia	**Ruptured membranes**: risk climbs	**Penicillin** before birth = GBS protection	**Early-onset** newborn sepsis

Spot It in Mom

- **Fever** (often the first sign)
- **Mom's HR > 100** and **baby's HR > 160**
- **Tender uterus**
- **Foul-smelling** or cloudy amniotic fluid
- **High WBC** count

Who's at Risk

- **Long rupture** of membranes
- **Long labor**
- **Many vaginal exams**
- **Internal monitors** (FSE, IUPC)
- **GBS positive**, BV, meconium in fluid

MEMORY HACK

Chorio clues: Hot · Fast · Foul · Tender

Hot	Fast	Foul	Tender
maternal **fever**	mom > 100, baby > 160	smelly, cloudy **fluid**	sore **uterus**

Nursing Actions: Mom

Goal: **treat the infection** + **protect baby**

STEP 1

Prevent

- **Limit vaginal exams** after membranes rupture
- Check **temp every 2 hrs** after rupture (per protocol)
- Sterile technique; give **GBS prophylaxis** when indicated

STEP 2

Treat

- **IV antibiotics** (often ampicillin + gentamicin)
- **Acetaminophen** for fever (fever raises baby's HR and O_2 needs)
- **Continuous fetal monitoring**
- Chorio alone is **not** a reason for a cesarean

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STEP 3

After Birth

- **Tell the pediatric/NICU team**: baby needs a sepsis workup
- Watch mom for **postpartum hemorrhage** (infected uterus contracts poorly) and **endometritis**

Group B Strep (GBS) Prevention

- **Swab at 36–37 weeks** (vagina + rectum). GBS lives normally in many healthy women but can infect baby at birth.
- **IV penicillin G** in labor (cefazolin or clindamycin/vancomycin if allergic). **At least 4 hrs before birth** to fully protect baby.
- Also treat if: GBS in urine this pregnancy, prior baby with GBS disease, or status **unknown** with preterm labor, rupture ≥ 18 hrs, or fever
- **Planned cesarean** with intact membranes and no labor → no GBS antibiotics needed

Newborn Sepsis: Spot It

Often just "**not acting right**":

- **Temperature instability**: often **low** temp, not fever
- **Poor feeding**, lethargy or irritability
- **Breathing trouble**: grunting, fast breathing, **apnea**
- **Low blood sugar**, jaundice, mottled skin, floppy tone

Early-onset (< 72 hrs) comes from **mom** (GBS, E. coli). **Late-onset** comes from the environment.

Newborn Sepsis: Act

- Draw **blood cultures before antibiotics**
- CBC, CRP; possibly lumbar puncture
- **IV ampicillin + gentamicin**; watch gentamicin **levels**, kidneys, and hearing
- Support: warmth, glucose, IV fluids, oxygen
- **Hand hygiene** is the #1 prevention

RED FLAGS · STOP AND CALL THE PROVIDER

Mom's **temp ≥ 38 °C** in labor · **Fetal HR > 160** that stays up · Newborn with **apnea**, **low temp**, or won't feed · Mom with **heavy bleeding** or a **soft (boggy) uterus** after birth

MUST-KNOW FOR THE NCLEX

- **Fever + fast heart rates + tender uterus + foul fluid** = chorioamnionitis.
- **GBS:** swab at 36–37 wks; IV penicillin in labor; ≥ 4 hrs before birth is best.
- Newborn sepsis is **sneaky**: low temp, poor feeding, and apnea are classic.
- **Cultures first, then antibiotics.**

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QUICK CHECK · TEST YOURSELF

1. The GBS swab is done at ____–____ weeks.
2. Give GBS antibiotics at least ____ hours before birth for full protection.
3. A septic newborn more often has a _____ temperature than a fever.
4. Before starting newborn antibiotics, draw _____.
5. After delivery, a mom with chorio is at higher risk for _____.

Answers: 1. 36–37 · 2. 4 · 3. low · 4. blood cultures · 5. postpartum hemorrhage (and endometritis)

For study use only. Always follow facility protocols, current guidelines, and provider orders.

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